

MCL quick reference

v2.0 specialist specialist guideline

MHA-MCL-2026-v2.0

Evidence and access cut-off: 28 July 2026

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Independent clinical review and pharmacy verification complete. Use with the current SmPC, local SACT protocol and jurisdiction-specific commissioning criteria.

PART 1 OF 3 — FIRST-LINE ORIENTATION

Before treatment

- Confirm integrated diagnosis and record blastoid or pleomorphic morphology, Ki-67 and TP53 status where technically feasible.
- Define symptoms, tempo, organ compromise, cytopenias, performance status, comorbidity, frailty and treatment goals.
- Separate clinical evidence from licence, NICE/HTA status and operational access.

Clinical state	Working orientation	Access boundary
Younger / treatment-fit	TRIANGLE changes the evidence architecture: the studied ibrutinib-containing non-ASCT strategy had mature efficacy; adding ASCT did not improve FFS.	GB-authorized regimen; NICE recommendation remains draft and negative. No final national entitlement at cut-off.
Older / transplant-ineligible	Choose through disease risk, frailty, renal, infection, cardiac, bleeding and interaction assessment. ECHO and ENRICH inform evidence but do not erase access constraints.	VR-CAP has TA370 in its exact population. Other modern regimens require separate live access confirmation.
TP53-mutated / high risk	Early specialist and trial discussion. Single-arm novel combinations show activity but do not prove comparative superiority or elimination of TP53 risk.	Do not present trial or early-access routes as routine commissioned treatment.
Observation	Selected asymptomatic low-volume, indolent or non-nodal disease may be observed after diagnostic confidence and MDT review.	Record review interval and objective reassessment triggers.

This quick reference does not contain all prescribing schedules and cannot replace the current SmPC or local SACT protocol.

PART 2 OF 3 — RELAPSED OR REFRACTORY PATHWAY

At every relapse

- Record every prior regimen and class, response depth and duration, treatment-free interval and reason for stopping.
- Distinguish covalent-BTKi intolerance from progression.
- Reassess tempo, tissue diagnosis where needed, performance status, organ function, infection history and trial eligibility.
- Refer early for cellular-therapy assessment where appropriate; account for manufacturing time and bridging risk.

State	Working pathway	Do not
Exactly one previous line	Ibrutinib TA502 or zanubrutinib TA1081 in England when exact live criteria are met.	Do not generalise the NICE one-line restriction to the broader licences.
Covalent-BTKi intolerance	Consider another clinically appropriate route according to retained sensitivity, toxicity, comorbidity and access.	Do not call intolerance resistant disease.
Covalent-BTKi progression	Assess CAR-T eligibility and trial options early. Consider non-covalent BTKi evidence only with a confirmed access route.	Do not infer routine pirtobrutinib funding from its MCL licence or from CLL access.
Post-BTKi, cellular therapy eligible	Brexu-cel remains TA677 CDF managed access when live national and panel criteria are met.	Do not describe TA677 as unrestricted baseline commissioning.
No routine national route	Use trial, early-access or exceptional pathways only when explicitly confirmed and governed.	Do not equate absence of a national route with absolute unavailability.

Do not use unadjusted cross-trial response rates to rank pirtobrutinib, cellular therapy, bispecific antibodies or investigational BCL2 inhibition.

PART 3 OF 3 — ACCESS AND SAFETY CONTROLS

Treatment	Exact population	England position at cut-off
VR-CAP / bortezomib combination	Previously untreated adults for whom haematopoietic stem-cell transplantation is unsuitable	Licensed and NICE-recommended in England for previously untreated adults for whom HSCT is unsuitable.
Ibrutinib monotherapy	Relapsed or refractory MCL after exactly one previous line for the NICE-funded population	Routinely funded in England after exactly one previous line when TA502 and live Blueteq criteria are met.
Zanubrutinib monotherapy	Relapsed or refractory MCL after exactly one previous line for the NICE-funded population	Routinely funded in England after exactly one previous line when TA1081 and live Blueteq criteria are met.
Brexucabtagene autoleucel	Adult relapsed or refractory MCL after at least two systemic lines including a BTK inhibitor	Available in England through TA677 Cancer Drugs Fund managed access when all live national eligibility and panel criteria are met.

Time-sensitive non-routine or unfinished routes

- TRIANGLE ibrutinib: licensed; current NICE recommendation is draft and negative, not final guidance.
- Acalabrutinib–BR: licensed for previously untreated adults not eligible for ASCT, but no demonstrated final positive national MCL route at cut-off.
- Acalabrutinib monotherapy: licensed for relapsed or refractory MCL not previously treated with a BTKi, but no demonstrated national England MCL commissioning route.
- Pirtobrutinib: licensed after previous BTKi treatment, but no demonstrated national England MCL commissioning route at cut-off.
- Liso-cel: licensed after at least two systemic lines including a BTKi, but no demonstrated national England MCL commissioning route at cut-off.
- Ibrutinib–venetoclax: no current MCL marketing authorisation; NICE appraisal suspended.
- Glofitamab and epcoritamab: do not extrapolate licences or funding from other lymphoma indications.

Pharmacy and supportive-care hold points

- Use the current SmPC and local SACT protocol for formulation, dose, schedule, modifications and administration.
- Complete infection, vaccination, viral-screening, interaction, bleeding, cardiac, renal and tumour-lysis assessment.
- Use dedicated CAR-T and bispecific toxicity-management pathways.
- Pharmacy verification was completed for the controlled release; use current SmPCs and local SACT protocols at the point of care.

Recheck current official national sources, SmPCs and local protocols before treatment because access and prescribing requirements may change.